

Progressive cicatrising conjunctivitis

Part 2 | Progression is an anatomical measurement

Record changing fornix depth, symblephara and lid mechanics.

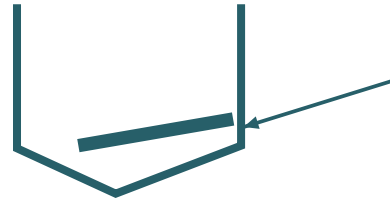
Preserved fornix

Serial anatomy, not redness alone, shows progression.



Deep conjunctival recess

Shortened fornix



Scar adhesion limits movement

Original schematic: a shortening fornix limits movement and disrupts tear distribution.

A single red-eye examination may underestimate cicatrization. Compare serial photographs and drawings of the fornices, caruncle, plica and lid margin. Document entropion, trichiasis and surface keratinisation. Scarring can advance with little visible inflammation, so a quiet-looking conjunctiva does not by itself establish stable disease.

Cicatrization is a phenotype, not one diagnosis

Mucosal and medication history determine the next branch.

Context	Reasoning direction
Progressive bilateral ocular scarring	Consider autoimmune mucous membrane pemphigoid
Severe acute mucocutaneous illness	Consider SJS/TEN and acute surface protection
Long-term topical drug exposure	Consider medicamentosa or pseudopemphigoid
Focal unilateral change	Reconsider local neoplasia or another mimic

Examine other mucosal sites and review every ocular medication. Autoimmune disease and toxic exposure can coexist. The correct question is whether an active scarring mechanism remains, not simply whether lubricants improve discomfort. Specialist coordination is important before biopsy or immunosuppression.

A negative biopsy does not settle a strong phenotype

Site, handling and repeat testing affect sensitivity.

- Coordinate biopsy site and transport with the laboratory.
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- Request direct immunofluorescence and complementary evaluation.
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- If negative, review sampling and consider another appropriate site.
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- Exclude mimics before accepting a clinical diagnosis.

European guidance recognises immunopathology-negative ocular MMP after appropriate investigation and exclusion of other causes. Severe ocular disease may lack demonstrable basement-membrane antibodies. Repeating the same poorly selected sample is not equivalent to a thoughtful repeat strategy; minimally inflamed or appropriate perilesional tissue and nonocular sites may improve yield.

SJS changes the whole ocular surface

Treat the acute mucosal injury before chronic sequelae become fixed.

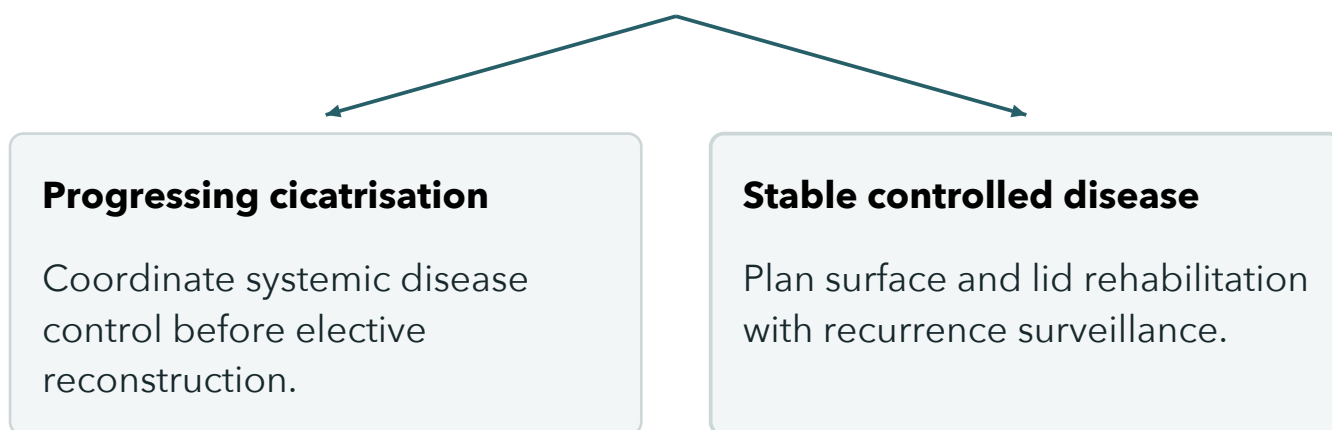
Acute finding	Longer-term consequence
Conjunctival epithelial loss	Adhesions and fornix shortening
Lid-margin injury	Keratinisation and blink trauma
Corneal epithelial loss	Infection, scarring and visual loss
Tear-system injury	Chronic surface instability

Early specialist ocular assessment is needed even when systemic illness dominates. Acute management may include amniotic membrane treatment when indicated and meticulous prevention of adhesions. Later care must address the lids, tear system and limbal function, rather than treating chronic dryness as an isolated tear-volume problem.

Reconstruction cannot replace inflammatory control

Repair the mechanism and the mechanics in the correct order.

Is the scarring process still active?



Elective lid or conjunctival surgery can provoke further inflammation in active MMP. Urgent threats such as severe lash trauma still need individualized protection while treatment takes effect. Long-term review should track anatomy, medication toxicity and systemic treatment adverse effects; apparent comfort alone is not an adequate disease endpoint.